

KANOLONE® Oral Base

Description: Homogeneous, light brown paste.

Composition: Each 100 g of paste contains Triamcinolone Acetonide 0.1 g

Pharmacodynamics: Kanolone® oral base is a topical corticosteroid. It shares anti-inflammatory, antipruritic, and anti-allergenic actions.

The mechanism of anti-inflammatory activity of topical corticosteroids is non-specific (ie., they act against most causes of inflammation including mechanical, chemical, microbiological and immunological).

Topically applied corticosteroids diffuse across cell membranes to interact with cytoplasmic receptors located in both the dermal and intradermal cells. The intracellular effects are similar to those that occur with systemically administered corticosteroids.

At the cellular level, corticosteroids appear to induce phospholipase A2 inhibitory proteins (lipocortins), thus depressing formation, release and activity of the endogenous mediators of inflammation such as prostaglandins, kinins, histamine, liposomal enzymes and the complement system.

When corticosteroids are applied to inflamed skin, they inhibit the migration of macrophages and leukocytes into the area by reversing vascular dilation and permeability. The clinical result is a decrease in edema, erythema and priorities.

By suppressing DNA synthesis, topically applied corticosteroids have an antimitotic effect on epidermal cells. This property is useful in proliferative disorders such as psoriasis, but also can be demonstrated in normal skin.

Pharmacokinetics : Topical corticosteroids can be absorbed from normal intact skin. Inflammation and/or other disease processes in the skin increase percutaneous absorption. Once absorbed through the skin, topical corticosteroids are handled through pharmacokinetic pathways similar to systemically administered corticosteroids.

Corticosteroids are bound to plasma proteins in varying degrees. Corticosteroids are metabolized primarily in the liver and are then excreted by the kidneys. Some of the topical corticosteroids and their metabolites are also excreted into the bile.

The emollient dental paste acts as an adhesive vehicle for applying the active medication to the oral tissues. The vehicle provides a protective covering which may serve to temporarily reduce the pain associated with oral irritation.

Indications: Kanolone® Oral Base is for local application. It is for adjunctive treatment and for the temporary relief of symptoms associated with oral inflammatory lesions and ulcerative lesions resulting from trauma.

Dosage and Administration: Apply a small dab of Kanolone® Oral Base to the lesion 3-4 times a day preferably after meals.

If significant repair or regeneration has not occurred in seven days, further investigation is advisable.

Contraindication : Kanolone® Oral Base is contraindicated in those patients with a history of hypersensitivity to any of the components of the preparation.

Because it contains a corticosteroid, Kanolone® Oral Base is contraindicated in the presence of fungal, viral, or bacterial infections of the mouth or throat.

Precautions:

General: Systemic absorption of topical corticosteroids see "Precautions-Pediatric Use" Conditions that augment systemic absorption include the application of the more potent steroids, use over large surface areas, prolonged use, and the addition of occlusive dressings.

Therefore, patients receiving a large dose of a potent topical steroid applied to a large surface area or under an occlusive dressing should be evaluated periodically for evidence of HPA axis suppression by using the urinary free cortisol and ACTH stimulation tests.

If HPA axis suppression is noted, an attempt should be made to withdraw the drug, to reduce the frequency of application, or to substitute a less potent steroid.

Recovery of HPA axis functions generally prompt and complete upon discontinuation of the drug. Infrequently, signs and symptoms of steroid withdrawal may occur, requiring supplemental systemic corticosteroids.

Children may absorb proportionally larger amounts of topical corticosteroids and thus be more susceptible to systemic toxicity (See "Precautions-Pediatric Use").

If irritation or sensitization develops, topical corticosteroids should be discontinued and appropriate therapy instituted.

In the presence of dermatological infections, the use of an appropriate or antibacterial agent should be instituted. If a favorable response does not occur promptly, the corticosteroid should be discontinued until the infection has been adequately controlled.

Patients with tuberculosis, peptic ulcer or diabetes mellitus should not be treated with any corticosteroid preparation without the advice of the patient's physician.

It should be borne in mind that the normal defensive responses of the oral tissues are depressed in patients receiving topical corticosteroid therapy. Virulent strains of oral microorganisms may multiply without producing the usual warning symptoms of oral infections.

The small amount of steroid released when Triamcinolone Acetonide dental paste is used as recommended makes systemic effects very unlikely; however, they are a possibility when topical corticosteroid preparations are used over a long period of time.

If significant regeneration or repair of oral tissues has not occurred in seven days, additional investigation into the etiology of the oral lesion is advised.

Pregnancy Category C: Corticosteroids are teratogenic in animals when administered systemically at relatively low dosages.

The more potent corticosteroids are teratogenic after dermal application in animals. There are no adequate and well-controlled

studies in pregnant women. Therefore, use during pregnancy only if the potential benefits outweigh the potential hazards to the fetus.

In pregnant patient, do not use extensively; do not use in large amounts or for prolonged periods of time.

Lactation : It is not known whether topical corticosteroids could result in sufficient systemic absorption to produce detectable quantities in breast milk. Systemic corticosteroids are secreted into breast milk in quantities not likely to have a deleterious effect on the infant. Nevertheless, exercise caution when administering topical corticosteroids to a nursing mother.

Pediatric Use: Children may be more susceptible to topical corticosteroid-induced hypothalamic-pituitary-adrenal (HPA) axis suppression and Cushing's syndrome than adults because of a larger skin surface area to body weight ratio.

HPA axis suppression, Cushing's syndrome and intracranial hypertension have occurred in children receiving topical corticosteroids. Manifestations of adrenal suppression include bulging fontanelles, headaches and bilateral papilledema.

Limit administration to the least amount compatible with effective therapy. Chronic corticosteroid therapy may interfere with the growth and development of children.

Drug Interactions: Corticosteroids reportedly may produce false-negative results with the nitrobluetetrazolium test for bacterial infections.

Side Effects / Adverse Reaction: The following local adverse reactions are reported infrequently with topical corticosteroids, but may occur more frequently with the use of occlusive dressings. These reactions are listed in an approximate decreasing order of occurrence.

Burning, Itching, Irritation, Dryness, Folliculitis, Hypertrichosis, Acne-form eruptions, Hypopigmentation, Peri-oral dermatitis, Allergic contact dermatitis, Maceration of the skin, Secondary infection, Skin atrophy, Striae, Miliaria.

Prolonged administration of Kanolone® Oral Base may elicit the adverse reactions known to occur with systemic steroid preparations; for example, adrenal suppression, alteration of glucose metabolism, protein catabolism, peptic ulcer activation, and others.

These are usually reversible and disappear when the hormone is discontinued.

Overdosage and Treatment: Topical corticosteroids can be absorbed in sufficient amounts to produce systemic effects (See Precautions). Recovery of HPA axis function is generally prompt and complete upon discontinuation of the drug.

Storage Conditions: Store below 30°C

After opening the tube, KANOLONE® Oral Base is stable for up to 30 days when stored at room temperature (23°C-27°C).

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Thai Reg. No. 1A 1754/28

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Shelf-life : 4 years

Packing/Pack size : 12 boxes of 1 tube per box with 5 grams of the paste per tube

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Manufactured by :



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